

Editor/Author Correspondence

Editor
2026-04-17
03:36 PM

Subject: [JMI ms#96541] Editorial Decision/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement"

Delete

Decision E2: (not accepted for JMIR Medical Informatics)

Dear Samuel Harrold,

Thank you very much for your submission to JMIR Medical Informatics. The handling Editor has completed review of the article. At this time, the manuscript cannot be considered for publication in JMIR Medical Informatics.

Although this seems to be an interesting manuscript, the handling Editor deemed the work not suitable for consideration in JMIR Medical Informatics because of one or more of the following reasons:

- (X) The manuscript may be publishable in principle but was rated as "low impact".
- (X) The manuscript seems to have methodological weaknesses/limitations.

We should also note that JMIR Medical Informatics is trying to position itself as a leading eHealth journal, so we have to be selective in what we publish. If we do not accept your manuscript then this is not necessarily an indication of the scientific quality of your work, but sometimes more a reflection of our editorial priorities and interests.

Transfer the manuscript to another JMIR Publications journal and re-instate
=====

For more information about JMIR Publications transfer and reinstatement process, please read: <https://support.jmir.org/hc/en-us/articles/115001714547-How-do-I-request-a-manuscript-transfer-to-another-journal-if-my-paper-was-rejected-or-I-am-unable-to-address-reviewer-comments->

JMIR Publications offers "portable peer-review" (also called cascading peer-review), meaning that manuscripts and their peer review comments, if available, can be transferred to other journals by the same publisher without going through a new peer-review process (for manuscripts rejected after at least one peer-review round). For newly submitted manuscripts that have not undergone peer-review, a new submission process is not needed with reinstatement and transfer, however, peer review would still be required. See <https://jmir.zendesk.com/hc/en-us/articles/115002462188> for more information.

Please let us know if you wish to reinstate and transfer the manuscript by contacting ed-support@jmir.org within four (4) weeks of this notification.

Note that different JMIR Publications journals have different Article Processing Fees. Please check <https://jmir.org/fees/article-processing-fees> to verify the APF for the new journal.

Thank you very much again for choosing JMIR Medical Informatics and giving us the opportunity to review your interesting work. We realize that this may be disappointing news, but we hope that you will continue to consider JMIR Medical Informatics or another JMIR Publications journal as an outlet for your high-quality research work.

Best regards,

Anastasia Shelomentseva
anna.shelomentseva@jmir.org
Editorial Assistant

=====

[Ms link for editor only: <https://medinform.jmir.org/editor/submissionReview/96541>
For section/guest editor:
<https://medinform.jmir.org/sectionEditor/submissionReview/96541>]
=====

Questions? Check out our knowledge base at <https://support.jmir.org/hc/en-us> !
To read our Privacy Policy, please see <https://support.jmir.org/hc/en-us/articles/360004220332>.

JMIR Medical Informatics has a new Journal Impact Factor of 3.8 (Source: Journal Citation Reports 2025 from Clarivate); see <https://medinform.jmir.org/announcements/590>

JMIR Medical Informatics is indexed in PubMed, PubMed Central, DOAJ, SCOPUS, and SCIE (Clarivate)

JMIR Medical Informatics <http://medinform.jmir.org/>

Editor
2026-04-20
03:22 PM

Subject: [MxM ms#96541] Suggest Transfer for Your Recently Submitted Manuscript

[Delete](#)

Dear Samuel Harrold:

Thank you very much for your message.

After editorial review, your manuscript was deemed better suited for another JMIR Publications journal, namely one of the following journals:

- (X) Online Journal of Public Health Informatics
- (X) Interactive Journal of Medical Research (i-JMR)
- (X) JMIR Bioinformatics and Biotechnology **

**Please note that if you agree to transfer to JMIR Bioinformatics and Biotechnology and the Editor of the journal accepts the manuscript for publication after peer review, then JMIR Publications can offer a one-time APF waiver for the authors to use only for this journal and for this manuscript.

*Please check <https://jmir.org/fees/article-processing-fees> to verify the APF for the new journal.

Manuscript URL: <https://manuscripts.jmir.org/author/submission/96541>
Username: samuel.harrold@gmail.com

Please let us know within ONE (1) week whether or not you agree to a manuscript transfer and, if so, we will seek approval of the transfer from the Editor-in-Chief of the respective journal.

Important: If we do not hear from you within ONE (1) week regarding your preferences about the above options, we will release the manuscript back to you so you can decide on the best next steps for your work. The submission will be marked in our system as "Rejected with option to reinstate" to the suggested journal. Then, you may choose to reinstate your paper to one of the suggested journal(s).

Note that while we await your response about transfer, your manuscript is reassigned to the Transfer Hub.

For more information about JMIR Publications transfer process, please read: <https://support.jmir.org/hc/en-us/articles/115002462188-Why-has-my-article-been-transferred-to-another-journal-or-a-transfer-has-been-suggested-and-what-are-my-options>

Please let us know how you would like us to handle your submission by replying to this message or contacting ed-support@jmir.org.

Best regards,

Anastasia Shelomentseva
anna.shelomentseva@jmir.org
Editorial Assistant

[Ms link for editor only: <https://manuscripts.jmir.org/editor/submissionReview/96541>
For section/guest editor:
<https://manuscripts.jmir.org/sectionEditor/submissionReview/96541>]

--

[Link for editor: <https://manuscripts.jmir.org/editor/submissionReview/96541>
Link for section editor:
<https://manuscripts.jmir.org/sectionEditor/submissionReview/96541>]

Editor
2026-04-23
04:21 PM

Subject: Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement

Samuel Harrold
Wed, Apr 22, 10:57 PM (17 hours ago)
to me, JMIR

Dear Anastasia Shelomentseva,

Thank you for your prompt response and for the transfer options provided.

I would like to proceed with a transfer to the Interactive Journal of Medical Research (i-JMR).

Please let me know if any additional information is needed to complete the transfer.

Best regards,
Samuel Harrold
samuel.harrold@yuimedi.com
Yuimedi, Inc.

Editor
2026-06-05
02:17 PM

Subject: [IJMR ms#96541] Editorial Decision/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement" [Delete](#)

Decision D: (major revision & re-review)

Dear Samuel Harrold,

Your manuscript (#96541) in Interactive Journal of Medical Research has now been reviewed. The relevant reviewer/editor comments are listed at the end of this email.

Interactive Journal of Medical Research cannot offer publication of the paper in its current form. Please address the issues mentioned below and submit your revised manuscript for review. Substantial revisions may be noted in the comments.

Please contact us as soon as possible if you are unable to fully address the identified issues, or if you are requesting transfer to another JMIR Publications journal. See <https://jmir.zendesk.com/hc/en-us/articles/115001714547>

The due date for your revision and responses to reviewers is 2026-07-03.

This deadline can be extended once (see <https://jmir.zendesk.com/hc/en-us/articles/115001333167>).

Revision & Resubmission Instructions

To upload a revised version of your manuscript, go to <https://www.i-jmr.org/author/submissionReview/96541> and do the following:

- 1) Revised Manuscript: Use the "Revised Ms Upload" file-upload function and upload a clean (non-tracked) revised Word document of your manuscript.
- 2) Author Response to Comments: Copy and paste point-by-point responses to reviewer/editor comments into the notification email to the editor. Please do not upload a separate file containing responses. There are no formatting options (e.g., bold, italics) in this field.
- 3) Upload a revised version with tracked changes (or highlighted sections of text where changes were made) as a supplementary file, in addition to providing a clean version under "Revised Ms". See <https://jmir.zendesk.com/hc/en-us/articles/115001400448> for details on how to respond to reviewer comments and upload a revised manuscript. Supplying a version of the manuscript with tracked changes facilitates the review process.

Note that it is the authors' responsibility to ensure adherence to JMIR Publications style and formatting, including final word count.

Author names and metadata

All co-authors should verify how their names and affiliations will be displayed by informing the submitting author if edits are required.

Also note that for the final publication (if accepted), metadata (author names and affiliations) will be taken from the database information you entered when submitting this article, NOT the manuscript file.

Thank you.

Best regards,

Matthew Balcarras, PhD
matt.balcarras@jmir.org
(Editor)

Interactive Journal of Medical Research
JMIR Publications

For status updates, please first read the FAQ (<https://support.jmir.org/hc/en-us/articles/115001358188>).

For scientific content questions only, please use the "Contact Editor" function on your manuscript link (<https://support.jmir.org/hc/en-us/articles/115001332927>).

For all other administrative or technical issues, or if you do not hear back from the academic editor on an urgent matter, please email ed-support@jmir.org with your manuscript number. Please note that academic editors will not respond to status queries or questions answered in the FAQ.

[Ms link for editor only: <https://www.i-jmr.org/editor/submissionReview/96541>
For section/guest editor: <https://www.i-jmr.org/sectionEditor/submissionReview/96541>]

EDITORIAL COMMENTS (Decision: D/major revision)

Thank you for your submission. Please see the comments below and provide a point by point response.

* Editorial and content comments *

- The central argument of your viewpoint is interesting and relevant. However the manuscript could be stronger by using better examples and interacting more with relevant and recent literature.

* Formatting/structure comments *

- All in-text references must be numbers in square brackets. Do not use the author-year system, round brackets, or superscripts. Follow our instructions for authors for references: <https://support.jmir.org/hc/en-us/articles/115001333067-How-should-references-be-formatted-Which-journal-style-should-I-choose-when-using-Endnote->

- Do not number your headings or use italics or bold for emphasis in place of subheadings. Ensure each section has more than one subheading; otherwise, remove it. All subheadings must be under Introduction, Methods, Results, and Discussion.

- (required) Funding Statement - Add a funding statement, distinct from the Acknowledgements. If no funding was involved, please state: "The authors declared no financial support was received for this work." More information: <https://support.jmir.org/hc/en-us/articles/360015982471-How-should-the-Acknowledgments-section-be-formatted->

- (required) Disclose whether generative AI was used in any portion of the manuscript generation: if used, it is mandatory to include a written statement in the manuscript, even if you disclosed use at the time of submission (e.g., in cover letter or submission form). If none, authors should attest to this in the response to comments. <https://support.jmir.org/hc/en-us/articles/13387268671771-Do-you-allow-the-the-use-of-ChatGPT-or-other-generative-language-models-and-how-should-this-be-reported-> Authors may optionally use this tool to generate a complete disclosure: <https://panbibliotekar.github.io/gaidet-declaration/index.html>

Instructions

Please upload a revised version with tracked changes (or highlighted sections of text where changes were made) as a supplementary file, in addition to providing a clean version under "Revised Ms". See <https://jmir.zendesk.com/hc/en-us/articles/115001400448> for details on how to respond to reviewer comments and upload a revised manuscript. Supplying a version of the manuscript with tracked changes facilitates the review process.

Note that it is the authors' responsibility to ensure adherence to JMIR Publications style and formatting, including final word count.

Addressing formatting issues and any remaining comments during the revisions period speeds up the production process in the event of acceptance.

Further Editorial / Peer-reviewer Comments:

External Peer-Review Report(s)

Reviewer Q:
General comments
=====

This paper addresses an important issue regarding the internalization of tacit healthcare analytics knowledge within healthcare organizations which usually leaves the organization due to high turnover rate among experts. Authors recommend a human in the loop mechanism to translate the queries of users into implementable SQL queries and run them through experts for validation and potential correction. The validated queries are then stored in a knowledge base for future references. While the concern regarding knowledge restoration is a valid challenge across all organizations, including healthcare, the proposed framework has certain major issues which require further attention.

Specific comments
=====

Major comments

1. The manuscript frequently claims low maturity level by referring to certifications such as HIMSS. It is worth noting that organizations primarily avoid such assessments due to high costs. Therefore, concerns about self selection bias requires attention. In addition, using such acronyms without proper explanation adds to complexity of the manuscript.
 2. The practicality and efficiency of the proposed framework depends on the volume, diversity and frequency of the queries generated by users. A deeper discussion around these aspects of the the query generation activities within a healthcare organization is needed. For instance, if an organization requires only 50 frequent queries, then the problem can be addressed via a proper documentation or even the use of an industry level knowledge base.
 3. One of the biggest concerns which requires further investigation is the whether the human in the loop can actually perform better than AI or LLMs. an empirical investigation of the performance of experts in detecting and correcting AI errors in this context can significantly add to the value of the manuscript. If that fails, the effectiveness of the model will be a subject of debate.
 4. Finally, if the data is constantly changing, as has been argued in the manuscript, then the tenure of the informatics experts might not be enough to address the maturity issue. in addition, trainings and awareness will be key players too.
- ...

Minor comments

5. please make sure to clearly explain the acronyms used throughout the manuscript, otherwise it will be complex to follow the flow of the paper.
6. some of the reference, specially those arguing about the tenure of healthcare IT workers, is more than 20 years old and should be replaced.

Please help us improve the quality of reviewers by providing a rating for each review using the URLs provided below (your ratings are anonymous and only seen by the editor):

TO RATE REVIEWER Q: CLICK ON ONE OF THE FOLLOWING URLS

1. very poor/misleading/wrong statements DO NOT USE REVIEWER AGAIN:

[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=603710&ro=1&ra=1&au=022cc970dc](https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=1&ra=1&au=022cc970dc)

2. poor review (e.g. very superficial, extremely short):

[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=603710&ro=1&ra=2&au=022cc970dc](https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=1&ra=2&au=022cc970dc)

3. below average (too short or various weaknesses):

[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=603710&ro=1&ra=3&au=022cc970dc](https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=1&ra=3&au=022cc970dc)

4. average (solid review with minor weaknesses):

[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=603710&ro=1&ra=4&au=022cc970dc](https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=1&ra=4&au=022cc970dc)

5. excellent / very useful review:

[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=603710&ro=1&ra=5&au=022cc970dc](https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=1&ra=5&au=022cc970dc)

Reviewer T:
General comments
=====

This viewpoint paper presents the author’s argument regarding persistent workforce turnover in the healthcare analytics domain and the associated challenges of retaining tacit knowledge. The author highlights that traditional apprenticeship models are increasingly ineffective, particularly given the short tenure of many professionals in healthcare settings. Although the issue itself is not novel, it remains a persistent and unresolved problem over time. To address this challenge, the author proposes leveraging artificial intelligence to capture and preserve tacit knowledge. The paper introduces a useful framework—human-in-the-loop knowledge governance (HITL-KG)—which aims to shift the locus of organizational knowledge from volatile human memory to durable semantic artifacts. These artifacts, termed “Validated Query Triples,” consist of a natural language intent, executable SQL, and accompanying rationale metadata, thereby enabling more sustainable knowledge retention and transfer.

Specific comments
=====

Major comments

Major Comments

1. Definition of Key Terms

Please clearly define the key terms used in the title—analytics maturity, workforce agility, and technical enablement. While their general meanings can be inferred, academic rigor requires that the author provide explicit definitions, ideally grounded in or adapted from existing literature. These are not novel constructs, and established definitions are likely available and should be cited to strengthen conceptual clarity.

2. Redundancy of Content

Some content appears repetitive. For example, statistics related to CIO tenure and turnover rates are introduced in the Introduction but are reiterated again in Section 2.1. Consider consolidating this information to avoid redundancy and improve the overall flow of the manuscript.

3. Clarification of Figure 1 (Framework Dynamics)

Figure 1 presents the architecture of the human-in-the-loop knowledge governance framework. However, the diagram currently depicts a static knowledge base and set of best practices. In reality, best practices are dynamic and evolve with new technologies and changing business rules. It would be more appropriate to reflect this iterative nature in the figure. Specifically, Step 7 (organizational memory) should feed back into Step 1 (knowledge base and best practices), following validation from prior steps, to illustrate continuous learning and system updating.

4. Use of Outdated Sources

Section 4.2 references a statistic from 2004. Given the rapidly evolving nature of healthcare analytics and workforce dynamics, this source is outdated. Please update with more recent evidence to ensure the argument reflects the current state of the field.

Minor Comment

• Please provide complete reference details for all citations, including volume, issue number, and page ranges. For example:
Ang, S., & Slaughter, S. (2004). Turnover of information technology professionals: The effects of internal labor market strategies. ACM SIGMIS Database: The DATABASE for Advances in Information Systems, [volume(issue)], pages.
<https://doi.org/10.1145/1017114.1017118>

Please help us improve the quality of reviewers by providing a rating for each review using the URLs provided below (your ratings are anonymous and only seen by the editor):

TO RATE REVIEWER T: CLICK ON ONE OF THE FOLLOWING URLS

1. very poor/misleading/wrong statements DO NOT USE REVIEWER AGAIN:

<https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=1&ra=1&au=ca9b670f90>

2. poor review (e.g. very superficial, extremely short):

<https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=1&ra=2&au=ca9b670f90>

3. below average (too short or various weaknesses):

<https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=1&ra=3&au=ca9b670f90>

4. average (solid review with minor weaknesses):

[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=477063&ro=1&ra=4&au=ca9b670f90](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=1&ra=4&au=ca9b670f90)

5. excellent / very useful review:
[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=477063&ro=1&ra=5&au=ca9b670f90](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=1&ra=5&au=ca9b670f90)

Questions? Check out our knowledge base at <https://support.jmir.org/hc/en-us>.
To read our Privacy Policy, please see [https://support.jmir.org/hc/en-
us/articles/360004220332](https://support.jmir.org/hc/en-us/articles/360004220332).

Interactive Journal of Medical Research's Journal Impact Factor is 2.2 (Source:
Journal Citation Reports 2025 from Clarivate); see [https://www.i-
jmr.org/announcements/585](https://www.i-jmr.org/announcements/585)

Interactive Journal of Medical Research is indexed in Sherpa/Romeo, DOAJ, ESCI,
PubMed, PMC, and EBSCO.

Interactive Journal of Medical Research: <http://www.i-jmr.org>

Author
2026-06-15
03:42 PM

Subject: [IJMR ms#96541] Author Response/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement" [Delete](#)

We thank the editor and both reviewers for their constructive and detailed comments. The revision strengthens the manuscript's engagement with recent peer-reviewed literature, adds explicit definitions of the three core constructs, addresses the scope and human-in-the-loop efficacy concerns with empirical evidence, and brings the formatting into compliance. In total we added 14 references spanning 1994-2026. Most are recent (2016-2026); the few older entries are foundational sources for the constructs and methods cited, each paired with a recent review: for example, Breu et al. 2002 (the origin of the workforce-agility construct) with Tessarini Junior & Saltorato 2021, and Tomaskovic-Devey et al. 1994 (foundational work on organizational survey non-response) with Halbesleben & Whitman 2013. Each comment is addressed below; section names refer to the revised manuscript.

=====

EDITORIAL AND CONTENT COMMENTS

=====

COMMENT (Editor): The central argument of your viewpoint is interesting and relevant. However the manuscript could be stronger by using better examples and interacting more with relevant and recent literature.

RESPONSE: We have substantially increased engagement with recent peer-reviewed literature throughout. Specifically, we added: cited scholarly definitions of the three title constructs (Reviewer T, comment 1); empirical evidence on the accuracy of human-in-the-loop oversight of AI-generated analytics (Reviewer Q, comment 3); database query-log evidence on query reuse and diversity (Reviewer Q, comment 2); methodological literature on self-selection and non-response bias in voluntary assessments (Reviewer Q, comment 1); and contemporary (2021-2025) workforce-turnover evidence. These additions are detailed in the responses below.

=====

FORMATTING AND STRUCTURE COMMENTS (Editor)

=====

COMMENT: All in-text references must be numbers in square brackets. Do not use the author-year system, round brackets, or superscripts.

RESPONSE: Done. The in-text citation style has been changed; references are now sequentially numbered and presented in square brackets (for example, [1], [2,3]), with no superscripts, author-year, or round brackets.

COMMENT: Do not number your headings or use italics or bold for emphasis in place of subheadings. Ensure each section has more than one subheading; otherwise, remove it. All subheadings must be under Introduction, Methods, Results, and Discussion.

RESPONSE: We have addressed each element:

- No headings are numbered.
- No section has exactly one subheading; each section has either no subheadings or two or more.
- The three short italic titles that previously labeled the sub-tables of the assessment rubric (Pillar 1, Pillar 2, Pillar 3) have been converted to proper descriptive subheadings, so italics are no longer used in place of subheadings. Remaining italics denote defined terms and list labels (emphasis), consistent with journal style.
- Regarding the requirement that all subheadings fall under Introduction, Methods, Results, and Discussion: this manuscript is a Viewpoint, and JMIR's own article-type instructions specify a different structure for Viewpoints than for Original Papers. Per "What are the article types for JMIR Publications journals?" (<https://support.jmir.org/hc/en-us/articles/115004950787-What-are-the-article-types-for-JMIR-Publications-journals>), the Viewpoint manuscript structure is: "Unstructured; using descriptive section headers that organize the work according to the content presented is strongly recommended. Headings such as 'Methods' and 'Results' are not allowed in Viewpoints." We have therefore used descriptive thematic headers throughout, as the Viewpoint article type requires, rather than IMRD headings. We would of course be glad to adjust if the editor prefers, recognizing that adopting an IMRD structure with Methods and Results headings would entail re-designating the article type away from Viewpoint.

COMMENT (required): Add a funding statement, distinct from the Acknowledgements.

RESPONSE: A Funding section is present and is distinct from the Acknowledgments. Because financial support was received, the "no financial support" statement does not apply; the Funding statement reads: "Yuimedi, Inc. provided financial support for

the author's time researching and writing this manuscript." This is consistent with the Conflicts of Interest disclosure of the author's role at Yuimedi.

COMMENT (required): Disclose whether generative AI was used in any portion of the manuscript generation.

RESPONSE: A generative AI disclosure is included in the manuscript (within Acknowledgments). It states that generative AI (Gemini CLI and Claude Code) was used to assist manuscript editing, language refinement, and literature search; that it was not used to generate research findings, data, or conclusions; and that the author conducted the research and verified all claims, citations, and AI-assisted text.

=====
REVIEWER Q
=====

COMMENT (Major 1): The manuscript frequently claims low maturity by referring to certifications such as HIMSS. Organizations primarily avoid such assessments due to high costs, so self-selection bias requires attention. In addition, acronyms are used without proper explanation.

RESPONSE: We added a self-selection-bias caveat in the Pillar 1 (Analytics Maturity) evidence: because HIMSS AMAM assessment is voluntary and self-selected, the reported figures likely over-represent analytics-committed organizations, so low maturity may be even more widespread than the counts suggest. This is supported by the organizational-survey methodology literature on non-response and self-selection bias [Halbesleben & Whitman 2013, Health Services Research; Tomaskovic-Devey et al. 1994, Administrative Science Quarterly]. We also broadened the maturity discussion beyond HIMSS by defining analytics maturity through the wider maturity-model literature (see Reviewer T, comment 1). Regarding cost as the specific mechanism of avoidance: we did not find an authoritative source attributing assessment avoidance specifically to cost, so we have framed the issue as self-selection bias grounded in the methodology literature rather than asserting an unsupported cost mechanism. On acronyms: all domain acronyms (HIMSS, AMAM, NL2SQL, EMR, EMRAM, LLM, ACO, EHR, CMS, CIO, DIKW, AACODS) are now defined at first use in the running text, and the Abbreviations list has been completed.

COMMENT (Major 2): The practicality of the framework depends on the volume, diversity, and frequency of queries. If an organization requires only 50 frequent queries, the problem can be addressed by documentation or an industry-level knowledge base.

RESPONSE: We added a scope-conditions discussion to the Comparison of Existing Approaches. Empirical query-log studies show that organizational analytical workloads are neither small nor static: more than half of analytical queries recur in large production workloads [Jindal et al. 2019], yet even small databases log thousands of distinct query strings [Kul et al. 2018]. A fixed set of documented queries therefore captures neither the large recurring core nor the evolving ad-hoc tail. HITL-KG addresses both regimes, reusing validated triples for recurring queries and capturing new ones at the point of use. This directly answers the "50 queries" objection: the recurring set is real but neither tiny nor static, and the diverse tail is exactly where documentation fails. The reviewer also raised the alternative of an industry-level knowledge base; the manuscript's Structural Barriers section addresses this, arguing that institution-specific business logic (local billing rules, coding conventions, and schema idiosyncrasies) is precisely what shared or industry-level knowledge bases cannot capture, as the failures of centralized efforts such as IBM Watson Health and Haven illustrate. HITL-KG captures this local logic rather than enforcing global standards.

COMMENT (Major 3): A central concern is whether the human-in-the-loop can actually perform better than AI or LLMs. An empirical investigation of experts detecting and correcting AI errors would significantly add value; otherwise the model's effectiveness is debatable.

RESPONSE: We agree this is the pivotal question. We have added empirical evidence to the Safety as Cognitive Forcing section showing that human oversight measurably improves AI-generated analytics: in a prospective study across six systematic reviews, large-language-model output verified by a human reviewer outperformed a human-only workflow (91.0% versus 89.0% accuracy) [Gartlehner et al. 2025, Annals of Internal Medicine]; in text-to-SQL specifically, an expert validation gate improved semantic correctness by 28.4% over end-to-end model output [Benzarti & Berrabah 2026]. We also cite balancing evidence that these gains are conditional on well-designed interaction [Ning et al. 2024]. Because this is a Viewpoint, we do not present an original empirical study; instead, we now frame HITL-KG explicitly as a prescriptive design whose comparative effectiveness is the subject of planned empirical validation, and the Implications section identifies the controlled evaluation (error rates, recovery time, and rework versus baseline tooling) as a priority for that

future work.

COMMENT (Major 4): If data is constantly changing, the tenure of informatics experts may not be enough to address the maturity issue; training and awareness will be key players too.

RESPONSE: We added a paragraph to Limitations acknowledging both points. HITL-KG complements rather than replaces workforce development: it decouples institutional knowledge from individual tenure but still presumes investment in training and analytics literacy [Konrad et al. 2022]. Moreover, precisely because retained expertise cannot manually keep pace with continual change in data models and business rules, knowledge must be externalized into curated artifacts; we also acknowledge that the validated query library itself requires active maintenance, which the Continuous Analytic Integration mechanism supports but does not eliminate.

COMMENT (Minor 5): Please clearly explain the acronyms used throughout the manuscript.

RESPONSE: Addressed as in comment 1: acronyms are now defined at first use, and the Abbreviations list is complete.

COMMENT (Minor 6): Some references, especially those on the tenure of healthcare IT workers, are more than 20 years old and should be replaced.

RESPONSE: The Workforce Agility evidence now relies on contemporary (2021-2025) data, including replacement-cost figures and current intent-to-leave and turnover statistics [for example, Rajamani et al. 2025; National Healthcare Retention report 2025]. The single remaining reference to the 2004 study (Ang & Slaughter) appears only once, in the Introduction, where it is explicitly framed as a two-decade-old benchmark whose continued use is itself evidence of the field's instability; the duplicate prior mention was removed.

=====
REVIEWER T
=====

COMMENT (Major 1): Please clearly define the key terms in the title (analytics maturity, workforce agility, technical enablement), ideally grounded in existing literature.

RESPONSE: We added explicit, cited definitions at the head of each evidence pillar. Analytics maturity is defined as an organization's progression in using data and quantitative models for fact-based decisions [Davenport & Harris 2007], operationalized through staged maturity models beyond the HIMSS model [Grossman 2018; Langer 2025]. Workforce agility is defined as a workforce's capacity for proactivity, adaptability, and resilience under change [Breu et al. 2002; Tessarini Junior & Saltorato 2021]. Technical enablement is defined as the technologies and practices that let non-specialist domain users access and analyze data without heavy reliance on central IT [Alpar & Schulz 2016; Bani-Hani et al. 2020].

COMMENT (Major 2): Some content is repetitive; for example, CIO tenure and turnover statistics are introduced in the Introduction and reiterated in Section 2.1. Consider consolidating.

RESPONSE: We removed the repeated statistics (the 53% CIO short-tenure figure and the 55% public-health-informatics intent-to-leave figure) from the "Broken Cycle" section, where they had duplicated the Introduction. These statistics are now stated once, at first mention in the Introduction, and the "Broken Cycle" section refers back to them while retaining only the figures unique to its argument.

COMMENT (Major 3): Figure 1 depicts a static knowledge base and best practices. Step 7 (organizational memory) should feed back into Step 1 (knowledge base and best practices) to illustrate continuous learning and system updating.

RESPONSE: We revised Figure 1 to add a feedback edge from Organizational Memory to the Knowledge Base, labeled "Curates best practices" (step 8). Validated query triples now update the ontologies and best practices that in turn provide context to future queries, closing a continuous-learning loop so that best practices evolve as new knowledge is validated rather than remaining static. The figure caption was updated to describe this loop.

COMMENT (Major 4): Section 4.2 references a statistic from 2004; please update with more recent evidence.

RESPONSE: Addressed together with Reviewer Q, comment 6. The 2004 statistic

was removed from that section, which now relies on contemporary (2021-2025) evidence; the only remaining mention is in the Introduction, explicitly framed as a historical benchmark.

COMMENT (Minor): Please provide complete reference details for all citations, including volume, issue number, and page ranges (for example, the Ang & Slaughter 2004 reference).

RESPONSE: The Ang & Slaughter (2004) reference is now complete: ACM SIGMIS Database, 35(3), 11-27, doi:10.1145/1017114.1017118. The newly added references were entered with full author lists, volume, issue, pages, and DOIs verified against Crossref. References that lack volume or page ranges are non-paginated sources (industry reports, institutional web resources, and press releases), cited with publisher and URL as appropriate.

=====

We thank the editor and reviewers again for their time. We believe the revisions materially strengthen the manuscript's evidentiary grounding, conceptual clarity, and formatting compliance, and we welcome any further guidance.

--
[Link for editor: <https://www.i-jmr.org/editor/submissionReview/96541>
Link for section editor: <https://www.i-jmr.org/sectionEditor/submissionReview/96541>]

Questions? Check out our knowledge base at <https://support.jmir.org/hc/en-us>.
To read our Privacy Policy, please see <https://support.jmir.org/hc/en-us/articles/360004220332>.

Interactive Journal of Medical Research's Journal Impact Factor is 2.2 (Source: Journal Citation Reports 2025 from Clarivate); see <https://www.i-jmr.org/announcements/585>

Interactive Journal of Medical Research is indexed in Sherpa/Romeo, DOAJ, ESCI, PubMed, PMC, and EBSCO.

Interactive Journal of Medical Research: <http://www.i-jmr.org>

Editor
2026-07-07
11:26 AM

Subject: [IJMR ms#96541] Editorial Decision/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement" [Delete](#)

Decision B: (minor revision)

Dear Samuel Harrold,

Your manuscript (#96541) in Interactive Journal of Medical Research has now been reviewed. The relevant reviewer/editor comments are listed at the end of this email.

Please address the issues mentioned below and submit your revised manuscript for editorial review.

A minor revision decision is not a guarantee of a final decision of acceptance. Please note that the editor may determine upon re-review that the paper is better suited for publication in another JMIR Publications journal and recommend a manuscript transfer, especially if critical issues remain unaddressed or are introduced in the course of further revision.

The due date for your revision and responses to reviewers is 2026-07-14.

This deadline can be extended once (see <https://jmir.zendesk.com/hc/en-us/articles/115001333167>).

Revision & Resubmission Instructions

To upload a revised version of your manuscript, go to <https://www.i-jmr.org/author/submissionReview/96541> and do the following:

- 1) Revised Manuscript: Use the "Revised Ms Upload" file-upload function and upload a clean (non-tracked) revised Word document of your manuscript.
- 2) Author Response to Comments: Copy and paste point-by-point responses to reviewer/editor comments into the notification email to the editor. Please do not upload a separate file containing responses. There are no formatting options (e.g., bold, italics) in this field.
- 3) Upload a revised version with tracked changes (or highlighted sections of text where changes were made) as a supplementary file, in addition to providing a clean version under "Revised Ms". See <https://jmir.zendesk.com/hc/en-us/articles/115001400448> for details on how to respond to reviewer comments and upload a revised manuscript. Supplying a version of the manuscript with tracked changes facilitates the review process.

Note that it is the authors' responsibility to ensure adherence to JMIR Publications style and formatting, including final word count.

Author names and metadata

All co-authors should verify how their names and affiliations will be displayed by informing the submitting author if edits are required.

Also note that for the final publication (if accepted), metadata (author names and affiliations) will be taken from the database information you entered when submitting this article, NOT the manuscript file.

Thank you.

Best regards,

Matthew Balcarras, PhD
matt.balcarras@jmir.org
(Editor)

Interactive Journal of Medical Research
JMIR Publications

For status updates, please first read the FAQ (<https://support.jmir.org/hc/en-us/articles/115001358188>).

For scientific content questions only, please use the "Contact Editor" function on

your manuscript link (<https://support.jmir.org/hc/en-us/articles/115001332927>).

For all other administrative or technical issues, or if you do not hear back from the academic editor on an urgent matter, please email ed-support@jmir.org with your manuscript number. Please note that academic editors will not respond to status queries or questions answered in the FAQ.

[Ms link for editor only: <https://www.i-jmr.org/editor/submissionReview/96541>
For section/guest editor: <https://www.i-jmr.org/sectionEditor/submissionReview/96541>]

EDITORIAL COMMENTS (Decision: B)

Thank you for the revisions to your manuscript. Please note the comments below and provide a point by point response.

Further Editorial / Peer-reviewer Comments:

Please see this link for details on manuscript length, the current version is too long: https://support.jmir.org/hc/en-us/articles/115004950787-What-are-the-article-types-for-JMIR-Publications-journals#h_01GB60593SQ0DCKPYTARYGRV6S

The reference list includes items with incorrect or invalid DOIs. Please review all references for accuracy, appropriateness and validity.

Please ensure that all tables and figures are included in the manuscript in proper format so that they can be viewed correctly. The current version has text that should probably be in tables but is not formatted in a way that can be viewed clearly.

----- snip -----

Reviewer Q:
General comments
=====

Thank you for addressing the reviewers' comments. The current draft has sufficient potential for publication in IJMR.
My only recommendation is that the proposed model has strong potential to be further expanded and empirically evaluated. Two particularly promising directions for future work are:

1. Empirical assessment of the human-in-the-loop component. Specifically, the authors could investigate whether incorporating a human expert into the validation process leads to a measurable improvement in the accuracy and quality of the generated knowledge base compared to a fully automated approach.
2. Comparative evaluation against existing models. The proposed framework could be compared with established approaches in a controlled or simulated setting to assess differences in the quality, completeness, and reliability of the generated knowledge base when responding to the same set of inquiries.

While I do not consider these analyses essential for the current manuscript, I believe they represent valuable avenues for future research that would further strengthen and validate the proposed framework.

Please help us improve the quality of reviewers by providing a rating for each review using the URLs provided below (your ratings are anonymous and only seen by the editor):

TO RATE REVIEWER Q: CLICK ON ONE OF THE FOLLOWING URLS

1. very poor/misleading/wrong statements DO NOT USE REVIEWER AGAIN:
<https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=2&ra=1&au=a922e144bb>
2. poor review (e.g. very superficial, extremely short):
<https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=2&ra=2&au=a922e144bb>
3. below average (too short or various weaknesses):
<https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=2&ra=3&au=a922e144bb>

ms=96541&re=603710&ro=2&ra=3&au=a922e144bb

4. average (solid review with minor weaknesses):
[https://www.i-jmr.org/rev/authorRating?](https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=2&ra=4&au=a922e144bb)
ms=96541&re=603710&ro=2&ra=4&au=a922e144bb

5. excellent / very useful review:
[https://www.i-jmr.org/rev/authorRating?](https://www.i-jmr.org/rev/authorRating?ms=96541&re=603710&ro=2&ra=5&au=a922e144bb)
ms=96541&re=603710&ro=2&ra=5&au=a922e144bb

Reviewer T:
General comments
Thank you for the revision. The author has satisfactorily addressed my concerns, and I have no further questions. One minor suggestion is to add figure captions. You currently have footnotes for each figure; please include captions as well.

Please help us improve the quality of reviewers by providing a rating for each review using the URLs provided below (your ratings are anonymous and only seen by the editor):

TO RATE REVIEWER T: CLICK ON ONE OF THE FOLLOWING URLS

1. very poor/misleading/wrong statements DO NOT USE REVIEWER AGAIN:
[https://www.i-jmr.org/rev/authorRating?](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=2&ra=1&au=b72fd30355)
ms=96541&re=477063&ro=2&ra=1&au=b72fd30355
2. poor review (e.g. very superficial, extremely short):
[https://www.i-jmr.org/rev/authorRating?](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=2&ra=2&au=b72fd30355)
ms=96541&re=477063&ro=2&ra=2&au=b72fd30355
3. below average (too short or various weaknesses):
[https://www.i-jmr.org/rev/authorRating?](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=2&ra=3&au=b72fd30355)
ms=96541&re=477063&ro=2&ra=3&au=b72fd30355
4. average (solid review with minor weaknesses):
[https://www.i-jmr.org/rev/authorRating?](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=2&ra=4&au=b72fd30355)
ms=96541&re=477063&ro=2&ra=4&au=b72fd30355
5. excellent / very useful review:
[https://www.i-jmr.org/rev/authorRating?](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=2&ra=5&au=b72fd30355)
ms=96541&re=477063&ro=2&ra=5&au=b72fd30355
-

Questions? Check out our knowledge base at <https://support.jmir.org/hc/en-us>.
To read our Privacy Policy, please see <https://support.jmir.org/hc/en-us/articles/360004220332>.

Interactive Journal of Medical Research's Journal Impact Factor is 3.3 (Source: Journal Citation Reports 2026 from Clarivate); see <https://www.i-jmr.org/announcements/709>

Interactive Journal of Medical Research is indexed in Sherpa/Romeo, DOAJ, ESCI, PubMed, PMC, and EBSCO.

Interactive Journal of Medical Research: <http://www.i-jmr.org>

Author
2026-07-11
02:42 PM

Subject: [IJMR ms#96541] Author Response/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement" [Delete](#)

We thank the editor and both reviewers for their time across two rounds of review. This revision addresses the three editorial items (manuscript length, reference DOIs, and table/figure formatting) and Reviewer T's suggestion to add figure captions. In the course of the reference audit we also made a small number of voluntary corrections, disclosed at the end of this letter, so that every citation and statistic matches its verified source. No reviewer-requested content from the first revision has been removed.

=====

EDITORIAL COMMENTS (Decision B)

=====

COMMENT (Editor): Please see this link for details on manuscript length, the current version is too long.

RESPONSE: The manuscript has been shortened to within the 5,000-word Viewpoint limit under JMIR's published counting method ("Which elements of my article are included in the word count?", which includes the title, abstract, keywords, body, table content and captions, figure captions, and end-matter sections, and excludes references, author metadata, figure image content, and multimedia appendices). By that method the previous submission measured approximately 5,430 words; the revision measures 4,990. The reduction was achieved by:

1. Moving the full Three-Pillar Assessment Rubric (three tables with Low/Medium/High scoring anchors and evidence citations) to a new Multimedia Appendix 2, following JMIR's guidance that lengthy tables are better suited to appendices. An abridged one-table summary of the nine indicators remains in the main text (Table 2), with a pointer to the appendix. All sources cited in the appendix rubric also remain cited in the main text and reference list.
2. Shortening the two figure descriptions into concise numbered captions with brief explanatory footnotes (see Reviewer T below).
3. Tightening prose throughout and shortening the abstract from 305 to 270 words. No substantive claim, statistic, construct, worked example, or reviewer-requested addition from the first revision was removed.

COMMENT (Editor): The reference list includes items with incorrect or invalid DOIs. Please review all references for accuracy, appropriateness and validity.

RESPONSE: We conducted a complete audit of the reference list. Every entry was checked for (a) DOI registration at doi.org, (b) agreement between the entry's title, authors, year, venue, volume, issue, and pages and the DOI's registered Crossref record, and (c) support for the specific claim cited to it, verified against the full text or publisher record wherever available. As a result:

- 8 incorrect or invalid DOIs were corrected (several were valid DOIs that resolved to a different article, typically citation-manager autofill errors; the corrected entries also received the right volume, issue, and page numbers, and in two cases the right journal or conference).
- DOIs were added to 9 entries that lacked them; 25 entries legitimately without DOIs (reports, news, and web resources) were verified by URL.
- 2 duplicate entries were removed.
- 7 references were removed or replaced after verification: two whose publisher site no longer serves any content, one student research report replaced by the established IEEE Spectrum account of the same case, one inaccessible thesis replaced by an already-cited peer-reviewed source that supports the same point, one press release replaced by the canonical scholarly source for the claim it supported, one removed from two sentences whose specific claims its content did not support, and one whose claim we re-grounded in two sources already in the reference list.
- Author names were corrected against publisher records in 29 entries (including wrong or incomplete given names, missing co-authors, and malformed name-list formatting that rendered incorrectly in the reference list), and bibliographic fields (volume, issue, pages) were completed throughout.

The revised list contains 84 references; every DOI resolves and matches its registered metadata.

COMMENT (Editor): Please ensure that all tables and figures are included in the manuscript in proper format so that they can be viewed correctly. The current version has text that should probably be in tables but is not formatted in a way that can be viewed clearly.

RESPONSE: We identified and fixed the root cause: the Word template style applied to tables defined no borders, so in the reviewed document the tables rendered as unaligned text without gridlines. The revision addresses this in three ways:

1. The table style now draws full gridlines with a heavier header rule, so both tables

display as clearly bounded tables in Word.

2. Tables are numbered with captions above them ("Table 1.", "Table 2.") and are cited in the text.

3. The three dense five-column rubric tables were moved to Multimedia Appendix 2 (see the length response above), leaving two compact tables in the main text. The appendix presents the rubric as three bordered tables with its own resolved reference list.

=====

REVIEWER T

=====

COMMENT: Thank you for the revision. The author has satisfactorily addressed my concerns, and I have no further questions. One minor suggestion is to add figure captions. You currently have footnotes for each figure; please include captions as well.

RESPONSE: Thank you; done. Each figure now carries a short numbered caption in the document's caption style ("Figure 1. Human-in-the-Loop Knowledge Governance (HITL-KG) architecture." and "Figure 2. The six-step Validated Query Cycle and its mapping to the three pillars."), followed by a brief explanatory footnote paragraph describing the steps shown. The previous long descriptions rendered as unnumbered text beneath each figure, which, as the reviewer observed, read as footnotes without captions; the revision provides both elements distinctly. The footnote for Figure 1 retains the description of the feedback loop from organizational memory to the knowledge base added in the first revision.

=====

REVIEWER Q

=====

COMMENT: Thank you for addressing the reviewers' comments. The current draft has sufficient potential for publication in IJMR. My only recommendation is that the proposed model has strong potential to be further expanded and empirically evaluated. Two particularly promising directions for future work are: 1. Empirical assessment of the human-in-the-loop component... 2. Comparative evaluation against existing models... While I do not consider these analyses essential for the current manuscript, I believe they represent valuable avenues for future research.

RESPONSE: Thank you for this assessment and for the constructive engagement across both rounds. We agree with both directions, and they correspond to the priority questions already identified in the manuscript's Conclusion: whether HITL-KG-mediated natural language to SQL workflows reduce error rates, recovery time, and rework compared with baseline tooling (the comparative evaluation), and how pillar scores correlate with observed continuity of analytics performance during turnover (the empirical assessment of the framework, including its human-in-the-loop component). These questions are the design basis for a planned companion empirical study using synthetic clinical data, and we look forward to reporting those results.

=====

ADDITIONAL CHANGES (disclosed for transparency)

=====

The reference audit prompted a small number of voluntary corrections beyond the requested items. None changes the manuscript's argument:

1. Construct attribution. The Introduction previously presented "Institutional Amnesia" as a term we defined. The term is established in public administration (Pollitt 2000, Prometheus; Stark and Head 2019, Journal of European Public Policy), and the underlying construct is studied in organization science as "organizational forgetting" (de Holan and Phillips 2004, Management Science; Rao and Argote 2006, already cited). The definition sentence now credits both literatures, and the paper's contribution is framed as specializing the construct to healthcare analytics. Three references were added.
2. Statistic precision. During full-text verification we tightened three statements to match their sources exactly: the Ang and Slaughter 2004 sentence now states the finding's actual scope (technical IT staff in organizations where IT is a support function, a category that includes healthcare delivery, had the lowest expected tenure at 2.9 years), rather than attributing the figure to healthcare IT staff specifically; an operational workforce statistic whose cited source did not contain it was replaced with figures from two peer-reviewed global meta-analyses already suited to the claim (global nurse turnover of 16%-18% annually; Ren et al. 2024, Journal of Nursing Management; Wu et al. 2024, Nursing Open); and the organizational-resilience sentence now claims only the no-consensus finding its cited systematic review supports.

3. Citation substitution. One reference added in the first revision to the Limitations section (Konrad and Saidizand 2022, an institutional repository thesis) became inaccessible at its source; it was replaced with Ledikwe et al. 2013 (Human Resources for Health), already in the reference list, which directly supports the same point about investment in training.

4. Generative AI disclosure (attestation). The disclosure in the Acknowledgments is unchanged: generative AI (Gemini CLI and Claude Code) was used to assist manuscript editing, language refinement, literature search, and reference verification; it was not used to generate research findings, data, or conclusions; the author conducted the research and verified all claims, citations, and AI-assisted text.

We thank the editor and reviewers again for their time. We believe the revision resolves the remaining editorial items while preserving all content from the peer-reviewed first revision, and we welcome any further guidance.

--

[Link for editor: <https://www.i-jmr.org/editor/submissionReview/96541>

Link for section editor: <https://www.i-jmr.org/sectionEditor/submissionReview/96541>]

Questions? Check out our knowledge base at <https://support.jmir.org/hc/en-us>.
To read our Privacy Policy, please see <https://support.jmir.org/hc/en-us/articles/360004220332>.

Interactive Journal of Medical Research's Journal Impact Factor is 3.3 (Source: Journal Citation Reports 2026 from Clarivate); see <https://www.i-jmr.org/announcements/709>

Interactive Journal of Medical Research is indexed in Sherpa/Romeo, DOAJ, ESCI, PubMed, PMC, and EBSCO.

Interactive Journal of Medical Research: <http://www.i-jmr.org>

Editor
2026-07-13
12:09 PM

Subject: [IJMR ms#96541] Editorial Decision/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement" [Delete](#)

Decision A: (accept/non-member)

Dear Samuel Harrold,

Thank you very much for your recent resubmission of a revised version of your article.

We have now completed the review of your article and are pleased to offer publication in Interactive Journal of Medical Research (IJMR, <https://www.i-jmr.org>).

If there are further comments from the reviewer or editor, they are listed HERE and/or at the end of this email.

--- Further editorial comments

If you are satisfied with our turnaround time and review process, it would help us enormously if you could give us a 5 star rating on Google here: <https://search.google.com/local/writereview?placeid=ChIJlavI58k0K4gR-wBqqGHwX7g>

---- end of editorial comments

From now on, please direct all production-related inquiries to prod-support@jmir.org. The production process will begin once your APF payment is received, at which point your production editor and copyeditor will reach out to you.

APF PAYMENT

The article processing fee (APF) for this article is US\$1,588.00.

Please note that all fees are exclusive of any applicable Value Added Tax (VAT), Goods and Services Tax (GST), or other local sales taxes. These will be added to the final invoice where applicable.

If your institution is registered for VAT, you may provide its VAT number in the Billing Address. Once validated, your invoice will be updated to remove the VAT charge. Please ensure the VAT number entered matches your institution's official billing details.

To pay by credit card (recommended), generate an invoice on the checkout page: <https://www.i-jmr.org/payment/billing/publication/96541> and follow the instructions included there.

Wire transfers and cheque payments are subject to a 7% handling fee. Be sure to select the correct method of payment on page 2 of the invoice. There is no handling fee for credit card payments.

Please note that your paper will only proceed to production once the APF, including any applicable handling fee, is paid in full.

For more information on payment methods, see <https://jmir.zendesk.com/hc/en-us/articles/115001267648>.

For our Payments FAQ, see <https://jmir.zendesk.com/hc/en-us/sections/115000389588-Payment>

PubMed Now!

Did you know that your accepted paper can be indexed in PubMed within 24 hours after this acceptance email? To request this (optional) service, select "PubMed Now!" when you pay the APF. For further details, see <https://jmir.zendesk.com/hc/en-us/articles/360008899632>

LICENSE TO PUBLISH (COPYRIGHT CLEARANCE) SIGNATURE REQUEST

All authors will receive an email from Dropbox Sign with a link to the Publication Form in order to collect electronic signatures. Publication is only possible once we have received signatures from all authors.

TRENDMD AND OTHER KNOWLEDGE DISSEMINATION ADD-ONS

When you make an APF payment, you can choose among several exciting knowledge translation tools and "add-ons", helping you to maximize impact and draw more readers and citations to your work, or even use your article as a fundraising tool:

- TrendMD - "advertises" your article on other publisher websites and leads to more visits and citations; see <http://www.jmir.org/content/trendmd> and <https://jmir.zendesk.com/hc/en-us/articles/115001380887> for details
- Promoted Tweets
- Author Ads - a banner ad that links readers to your organizations'/departments/research groups' homepage, product page, recruitment page
- PubMed Now! - index your accepted paper within 24 hours - <https://jmir.zendesk.com/hc/en-us/articles/360008899632>
- Visual Abstract - We can create a TOC image with an infographic for you <https://support.jmir.org/hc/en-us/articles/360053387451-What-is-a-Visual-Abstract->

You can select these and other items on the check-out page at <https://www.i-jmr.org/payment/billing/publication/96541>

For more information about our Knowledge Translation tools and add-ons, see <https://jmir.zendesk.com/hc/en-us/sections/115000441248>.

JMIR Publications partnered with Storytap to provide you with an innovative way to showcase your research. Using Storytap, you create a short video highlighting your work, which expands its reach and impact.

* Be on the lookout for an invitation from the JMIR Publications Team – take this FREE chance to create a video to enhance the visibility of your research.

* Please keep an eye out for the Storytap invitation email – it's not spam! For more about Storytap: <https://support.jmir.org/hc/en-us/articles/29595102581275>

FIGURES AND APPENDICES

We hope you can upload a nice TOC image, e.g., showing the use of your mobile app in context. See <https://support.jmir.org/hc/en-us/articles/115001301407-What-makes-a-good-TOC-image-> for instructions.

FIGURE AND MULTIMEDIA APPENDIX CAPTIONS FOR THE FINAL PUBLICATION ARE NO LONGER GENERATED FROM THE MANUSCRIPT. INSTEAD, THEY ARE GENERATED FROM OUR DATABASE (what you enter in the field "caption" when you upload a figure or appendix).

ALL Figures intended for publication MUST appear under the "Figures" heading on the copyediting/layout page <https://www.i-jmr.org/author/submissionEditing/96541> (or <https://www.i-jmr.org/author/submissionReview/96541>)

ALL Multimedia Appendices intended for publication MUST appear under the "Multimedia Appendix" heading on the copyediting/layout page (if not, the copyeditor should upload them or instruct the author to upload them).

VERIFY THAT ALL FIGURES AND ALL MULTIMEDIA APPENDICES APPEAR on the copyediting/layout page <https://www.i-jmr.org/author/submissionEditing/96541> (or <https://www.i-jmr.org/author/submissionReview/96541>)

Click on EDIT to check/edit the figure or multimedia appendix caption and edit the caption. Please note: for figures/appendices, the caption will be displayed in the final publication, preceded by "Figure #." or "Multimedia Appendix #. " (where # is the number). Do not include a figure/appendix number in the caption field; this will be generated automatically.

Please make sure that you have uploaded all figures in the highest resolution possible (preferably lossless PNG files).

In addition, we encourage authors to include a multimedia appendix, for example, a powerpoint-file from a presentation of the study and/or a PowerPoint or pdf file showing screenshots of your intervention. If you haven't done so already, you can upload such a file at <https://www.i-jmr.org/author/submissionReview/96541>

REFERENCES:

Make sure your references are properly formatted before we send the file for

copyediting/typesetting. We will be using a software called RefCheck to correct your references automatically, but this software will create more work than it saves if the references are not properly formatted or at least have a PMID beforehand.

See our FAQ at <https://jmir.zendesk.com/hc/en-us/articles/115001333067>.

If you used a reference management system (like Endnote), make sure to remove all field codes from your manuscript (select all text, then press Ctrl+Shift+F9 or Cmd+6 to unlink all fields).

For best results, simply write the PubMed Identifier (PMID) behind each reference (e.g., PMID:12345 - without brackets), or append a Medline link to the abstract (the link contains the PMID as well). For books, add the ISBN number after the reference, e.g., ISBN:12342345X.

Make sure to update references previously cited as "in press" or "forthcoming" if they are now published (add the PMID or DOI if known)

After making your final corrections to the references (and checking/removing author information - see below), you can upload your final manuscript:

<https://www.i-jmr.org/author/submissionReview/96541>

PLEASE DO SO AS SOON AS POSSIBLE (ideally within 24-48 hours) AFTER RECEIVING THIS EMAIL, AND BEFORE PAYING THE APF, SO AS TO AVOID OUR SENDING THE WRONG FILE TO THE COPYEDITOR.

ORCIDs

ORCIDs are mandatory for all authors, including your co-authors - see also: <https://jmir.zendesk.com/hc/en-us/articles/115001351808>.

Before we can publish the paper, we need the Open Researcher and Contributor ID (ORCID) of all author(s) of the manuscript.

ORCID is an open, non-profit, community-based effort that has been created by a number of leading research funders, universities, and publishers to solve the name ambiguity problem in scholarly communications by creating a registry of persistent unique identifiers for individual researchers and an open and transparent linking mechanism between ORCID, other ID schemes, and research objects such as publications, grants, and patents.

The ORCID is now also submitted and searchable on PubMed. If we do not have your ORCID on file, users may not be able to associate this publication with you or your coauthors, which may disadvantage you in future research assessments or grant funding.

To create an ORCID, please follow the three steps:

Go to [ORCID.org](https://orcid.org) and create your account.

Once you have one, please enter your ORCID into your JMIR user profile at <http://www.jmir.org/user/profile>. See <https://support.jmir.org/hc/en-us/articles/115003479088> for instructions.

The final step is to enter the ORCID (and those of your co-authors) into the metadata form of the current manuscript, which can be found at <https://www.i-jmr.org/author/viewMetadata/96541>.

If you are the SUBMITTING author, please contact your coauthors and ask them for their ORCID. If they have a JMIR Publications account and entered the ORCID in their profile, and if this profile has the same email address as the email addresses entered in the manuscript submission, the submission form may already have been automatically populated, but in most cases, the submitting author needs to enter them in the metadata form above. All coauthors need to have an ORCID; there are no exceptions (unless they are deceased). See <https://support.jmir.org/hc/en-us/articles/115000606771> for instructions on how to enter the ORCIDs of your coauthors.

If you are a COAUTHOR (not the submitting author but a coauthor cc'ed on this), please also follow steps 1 and 2 above to create an ORCID and enter it into your JMIR profile. *** To associate your ORCID with the recently accepted manuscript, please provide the submitting author with your ORCID and request that they complete your information. ***

You can disregard the instructions above if you already entered ORCIDs on submission.

Please also take a moment to ensure that your other metadata (spelling of your name, degrees, affiliation) is correct, as we cannot make changes after publication. See below for further instructions.

VERIFY METADATA (AUTHOR NAMES/AFFILIATIONS) [SEE ALSO ATTACHMENT!}

To be correctly credited as an author is extremely important for scholarly authors (including your coauthors). Please use the time between now and copyediting to ascertain that all authors and their affiliations will be correctly listed in the final publication.

When you submitted your article, you were asked to enter all authors' names, degrees, and affiliations. A PDF PREVIEW of these data is attached.

It is important to understand that this information FROM THE METADATA FORM (NOT THE AUTHORSHIP INFORMATION FROM YOUR MANUSCRIPT) is used for the final publication, will be submitted to bibliographic databases such as Medline, and will be used to build our table of contents. It is therefore important that you check whether the names of all authors (including your co-authors) are correctly spelled (including middle names/initials etc.) and that their affiliations are correct as well (copy and paste from the final manuscript, if necessary). We also need the email addresses of your co-authors on file (they will not be published, but we need to have them on file).

See <https://jmir.zendesk.com/hc/en-us/articles/115002773827> for details on how to enter the metadata.

The submitting author (not the coauthors) can download a PDF PREVIEW showing how the current list of authors and authorship information will appear in the final manuscript from the following URL (login first):

<https://www.i-jmr.org/front/pdf/96541/1>

(this PDF is generated dynamically - press reload after making changes to the metadata)

To correct the data, please click on EDIT METADATA in the SUMMARY tab of your submission to verify that the metadata are correct and complete.

The URL to edit the metadata is

<https://www.i-jmr.org/author/viewMetadata/96541>.

After saving, you can go back to <https://www.i-jmr.org/front/pdf/96541/1> to generate a new PDF preview.

Repeat this process until all authors have approved how the authorship information will appear in the final paper.

To avoid confusion with author names and affiliations from your final manuscript file, please remove author names and affiliations from your final manuscript file and add a comment for the copyeditor (using the Word comment function) stating "I and my coauthors have approved the authorship information as rendered on <https://www.i-jmr.org/front/pdf/96541/1>".

Then upload your final manuscript (with author names and affiliations removed) to: <https://www.i-jmr.org/author/submissionReview/96541>

Citing this article as "in-press"

If you are already working on follow-up publications (hopefully to be submitted to Interactive Journal of Medical Research as well!), feel free to cite this article as "forthcoming" (or "in press").

While we usually try to publish accepted articles in the current or upcoming issue, we cannot provide you with the exact bibliographic information (volume, issue, article number) at this time.

YOU MAY HOWEVER USE THE DOI (Digital Object Identifier) TO CITE THE ARTICLE, USING YOUR MANUSCRIPT NUMBER ms#96541 (see subject line). The DOI consists of a prefix identifying IJMR (10.2196/) and your manuscript number:

Citation suggestion:

Author name(s). Title. Interact J Med Res (forthcoming). doi:10.2196/96541

<http://dx.doi.org/10.2196/96541>

(note that the above URL linking to the so-called DOI resolver produces an error message at the moment when using your ms#, but immediately after the article is published (on the same day), it will point to the published article URL).

Using the DOI for in-press articles is a widely accepted standard, so even if you do not submit your follow-up paper to Interactive Journal of Medical Research you should use this format.

PRESS RELEASE

Now is the appropriate time to contact your institutional communications / PR department to draft a press release on your forthcoming publication in the Interactive Journal of Medical Research. Some studies generate considerable media interest (as evidenced in our "In the news" website section). We will be happy to assist in this process if you provide us with a contact email. We also encourage you to send us your draft press release, should you have one.

For more information, please see our Press Release FAQ at <https://jmir.zendesk.com/hc/en-us/articles/115001301567>.

Finally, we should also note that Interactive Journal of Medical Research is trying to position itself as a leading journal. Thus we are highly selective in what we publish (the acceptance rate is currently around 25%). We only publish papers that we think have a considerable impact on the field and will be cited often.

We look forward to seeing your paper published. Thank you very much again for choosing Interactive Journal of Medical Research and allowing us to review your interesting work.

Best regards,

Matthew Balcarras, PhD
matt.balcarras@jmir.org
(Editor)

[Ms link for editor only: <https://www.i-jmr.org/editor/submissionReview/96541>
For section/guest editor: <https://www.i-jmr.org/sectionEditor/submissionReview/96541>]

Questions? Check out our knowledge base at <https://support.jmir.org/hc/en-us>.
To read our Privacy Policy, please see <https://support.jmir.org/hc/en-us/articles/360004220332>.

Interactive Journal of Medical Research's Journal Impact Factor is 3.3 (Source: Journal Citation Reports 2026 from Clarivate); see <https://www.i-jmr.org/announcements/709>

Interactive Journal of Medical Research is indexed in Sherpa/Romeo, DOAJ, ESCI, PubMed, PMC, and EBSCO.

Interactive Journal of Medical Research: <http://www.i-jmr.org>

Close